

Belmar — Semaglutide

GLP-1 Provider Reference

Weight Loss Program · KORB Health Group · korb-glp1-data.js v2.14 · built 2026-09-06

Everything needed to prescribe Belmar Semaglutide, complete on its own. Values are copied literally into Tebra — do not paraphrase, and do not adjust quantity, refill or days supply.

Belmar Pharmacy at a glance

Status	Active
Preferred states	CA
Ships to	AL, AK, AZ, AR, CA, CO, CT, DE, DC, FL, GA, HI, ID, IL, IN, IA, KS, KY, LA, ME, MD, MA, MI, MN, MS, MO, MT, NE, NV, NH, NJ, NM, NY, NC, ND, OH, OK, OR, PA, RI, SC, SD, TN, TX, UT, VT, VA, WA, WV, WI, WY
CANNOT ship to	No hard exclusions
Order via	Tebra Compound
Billing	Bill to KORB Health Group, ship to patient
Dispensing address	Belmar Pharmacy — ARIZONA location, 12012 N 111th Ave, Youngtown, AZ 85363-1339

Check the address before sending

Belmar has several locations across the US. KORB uses the ARIZONA address. Confirm the Arizona address is the one selected in Tebra before sending — another Belmar location will be wrong.

Discouraged outside CA

Belmar is the California pharmacy. Patients are routed here for California and kept on the other compounding pharmacies elsewhere. Selecting Belmar outside California is allowed but should be a deliberate exception.

Compounded preparation

KORB dispenses a COMPOUNDED preparation, not the FDA-approved branded product. The molecule is approved; this specific formulation is not, and compounded products are not reviewed by the FDA for safety, efficacy or quality. Counsel accordingly and document that the distinction was explained.

Known limitation — 1.0 mg, 1.7 mg, 2.4 mg

These three doses use a 5 ml vial that yields 5.00, 7.35 and 5.21 weekly doses against a 4-dose ceiling. A patient who finishes the open vial rather than discarding it draws from it past the 28-day in-use limit, and gets 1.0 to 6.7 doses more than intended.

Mitigation. Every compounded injectable sig now carries "Discard after 4 doses or 28 days." On these three doses that instruction is the whole mitigation rather than a reminder - the vial physically holds a 5th dose, so if the line is dropped the limitation becomes an uncontrolled 28-day breach. Applied program-wide on 2026-09-05 at Don's direction, for consistency: harmless where a vial empties before four doses, protective where it does not.

What this still leaves. Counseling does not fully solve it. The reason this was raised in the first place is that patients do not discard a vial with drug left in it. Expect extra doses at these three tiers and expect the short-supply complaint at the next escalation, particularly moving off 1.7 mg where the spare is 6.7 doses.

For Operations. A patient reporting they "ran out early" after stepping up from 1.7 mg is almost certainly comparing against extra doses they should not have had. Treat it as expected rather than as a dispensing error.

Accepted by Don on 2026-09-05 · BELMAR-SEMA-VIAL-OVERSIZE

Before you prescribe

- Carries both semaglutide and tirzepatide. All new California starts go here regardless of drug.
- SINGLE FILL as of 2026-09-05. Belmar now ships the full 8-week supply at one time for both semaglutide and tirzepatide. The split fill is retired. Belmar no longer differs from the other pharmacies on fill structure.
- Belmar 8-week now bills on the standard 8-week charge code. The Belmar-specific codes are retired — they existed only to trigger the second fill, and there is no second fill.
- Semaglutide 2.5 mg / 1 mg / ml is used for 1.7 and 2.4 mg doses; 1 mg / 1 mg / ml for 0.25, 0.5 and 1.0 mg doses.
- SYRINGES GO IN THE DIRECTIONS, NOT PHARMACY INSTRUCTIONS. Belmar reads the patient directions field. If the insulin-syringe note is moved to Pharmacy Instructions they may not see it and will not ship syringes. Keep "(Include one pack of insulin syringes)" at the end of every Belmar sig.

Vials dispensed

Dose	Units	Concentration	4-week	8-week
0.25 mg	25 units	1 mg/1 mg/ml	1 ml x 1 vial	1 ml x 2 vials
0.5 mg	50 units	1 mg/1 mg/ml	1 ml x 2 vials	1 ml x 4 vials
1.0 mg	100 units	1 mg/1 mg/ml	5 ml x 1 vial	5 ml x 2 vials
1.7 mg	68 units	2.5 mg/1 mg/ml	5 ml x 1 vial	5 ml x 2 vials
2.4 mg	96 units	2.5 mg/1 mg/ml	5 ml x 1 vial	5 ml x 2 vials

Tebra — copy these values literally

Do not paraphrase, and do not adjust quantity, refill or days supply.

Belmar — Semaglutide / B-12

Tebra Compound

Same for every dose

Allow Substitution	Yes – select Allow Substitution
Unit	ml
Reason for Compounding	N/V mitigation & flexibility
Pharmacy Instructions	Bill to KORB Health Group and ship to the patient. Custom Rx for N/V mitigation and dosing flexibility. Allergies:

0.25 mg — 4-week

Drug Formulation	Semaglutide/B-12 1mg/1mg/ml
Name	BELMAR – Semaglutide 0.25 mg – 4-Week Supply
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	INJECT 0.25 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

0.25 mg — 8-week

Drug Formulation	Semaglutide/B-12 1mg/1mg/ml
Name	BELMAR – Semaglutide 0.25 mg – 8-Week Supply
Quantity	2
Refill	0
Days Supply	56
Patient Instructions	INJECT 0.25 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

0.5 mg — 4-week

Drug Formulation	Semaglutide/B-12 1mg/1mg/ml
Name	BELMAR – Semaglutide 0.5 mg – 4-Week Supply
Quantity	2
Refill	0
Days Supply	28
Patient Instructions	INJECT 0.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

0.5 mg — 8-week

Drug Formulation	Semaglutide/B-12 1mg/1mg/ml
Name	BELMAR – Semaglutide 0.5 mg – 8-Week Supply
Quantity	4
Refill	0
Days Supply	56
Patient Instructions	INJECT 0.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

1.0 mg — 4-week

Drug Formulation	Semaglutide/B-12 1mg/1mg/ml
Name	BELMAR – Semaglutide 1.0 mg – 4-Week Supply
Quantity	5
Refill	0
Days Supply	28
Patient Instructions	INJECT 1 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

1.0 mg — 8-week

Drug Formulation	Semaglutide/B-12 1mg/1mg/ml
Name	BELMAR – Semaglutide 1.0 mg – 8-Week Supply
Quantity	10
Refill	0
Days Supply	56
Patient Instructions	INJECT 1 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

1.7 mg — 4-week

Drug Formulation	Semaglutide/B-12 2.5mg/1mg/ml
Name	BELMAR – Semaglutide 1.7 mg – 4-Week Supply
Quantity	5
Refill	0
Days Supply	28
Patient Instructions	INJECT 1.7 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

1.7 mg — 8-week

Drug Formulation	Semaglutide/B-12 2.5mg/1mg/ml
Name	BELMAR – Semaglutide 1.7 mg – 8-Week Supply
Quantity	10
Refill	0
Days Supply	56
Patient Instructions	INJECT 1.7 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

2.4 mg — 4-week

Drug Formulation	Semaglutide/B-12 2.5mg/1mg/ml
Name	BELMAR – Semaglutide 2.4 mg – 4-Week Supply
Quantity	5
Refill	0
Days Supply	28
Patient Instructions	INJECT 2.4 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

2.4 mg — 8-week

Drug Formulation	Semaglutide/B-12 2.5mg/1mg/ml
Name	BELMAR – Semaglutide 2.4 mg – 8-Week Supply
Quantity	10
Refill	0
Days Supply	56
Patient Instructions	INJECT 2.4 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

Belmar — Oral Semaglutide FastSL

Tebra Compound

Same for every dose

Drug Formulation	Semaglutide FastSL 1mg
Allow Substitution	Yes – select Allow Substitution
Unit	each
Reason for Compounding	N/V mitigation & flexibility
Pharmacy Instructions	Bill to KORB Health Group and ship to the patient. Custom Rx – no FDA-approved or commercially available equivalent.

0.5 mg · FastSL Tablet #45 — 90-day

Name	BELMAR — Oral Semaglutide 0.5 mg — 90 Day Supply
Quantity	45
Refill	0
Days Supply	90
Patient Instructions	DISSOLVE 1/2 TABLET UNDER TONGUE DAILY ON EMPTY STOMACH. NO FOOD OR DRINK 15 MIN AFTER.

1 mg · FastSL Tablet #90 — 90-day

Name	BELMAR — Oral Semaglutide 1 mg — 90 Day Supply
Quantity	90
Refill	0
Days Supply	90
Patient Instructions	DISSOLVE 1 TABLET UNDER TONGUE DAILY ON EMPTY STOMACH. NO FOOD OR DRINK 15 MIN AFTER.

Pricing and charge codes**Belmar — Semaglutide / B-12 — 4-week**

Program	Price	Charge code
4-week — Full price — website rate	\$269	FITSema001
4-week — Discounted rate	\$199	Operations will provide the charge code at scheduling
4-week — All others	Varies	Operations will provide the charge code at scheduling

KORB Get Fit Now Semaglutide Standard

One rate across all three categories. No per-partner variation.

Partner-funded, partially covered, and other arrangements. Highly variable and changes often. Operations confirms the figure and the code at scheduling. Do not quote a price from this tool.

Belmar — Semaglutide / B-12 — 8-week

Program	Price	Charge code
All 8-week semaglutide, every patient, every pharmacy	\$349	FITSemaMNT

No discounted rate on 8-week. Corporate, friends and family, and military patients all pay \$349 for 8-week. The \$199 rate is 4-week only.

Belmar — Oral Semaglutide FastSL — 90-day

Program	Price	Charge code
FastSL Tablet #45	\$299	FITSem0r190

Includes: Telehealth visit, medication, shipping and supplies. Never quote pricing to a patient without confirming with Operations.

Clinical reference — Semaglutide — GLP-1 receptor agonist

Indications

- Chronic weight management and long-term maintenance of weight reduction in adults with obesity, or overweight with at least one weight-related comorbid condition
- Improvement of glycemic control in adults with type 2 diabetes mellitus
- Reduction of cardiovascular risk in adults with type 2 diabetes and established cardiovascular disease
- Reduction of kidney disease progression and cardiovascular death in adults with type 2 diabetes and chronic kidney disease
- Treatment of noncirrhotic MASH with moderate-to-advanced liver fibrosis (F2-F3) in adults

What KORB treats

KORB treats weight loss only. Every other indication above stays with the patient's PCP or specialist - KORB does not manage type 2 diabetes, cardiovascular risk, chronic kidney disease or MASH, and does not order or monitor labs for them. A patient carrying one of those diagnoses is acceptable on the weight-loss program provided that care continues elsewhere.

Identifying the appropriate candidate

Age. Adults 18 years and older. KORB does not treat patients under 18.

BMI

- BMI greater than 25 is overweight
- BMI greater than 30 is obese

Diabetes Note. KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Definition

Long-acting glucagon-like peptide-1 (GLP-1) receptor agonist. A 31-amino-acid analogue with roughly 94% homology to human GLP-1, acylated with a C18 fatty diacid that binds albumin and extends the half-life to about one week, allowing once-weekly dosing. FDA-approved as Ozempic and Rybelsus for type 2 diabetes and as Wegovy for chronic weight management.

Mechanism

- Agonises the GLP-1 receptor.
- Glucose-dependent insulin secretion from pancreatic beta cells.
- Suppression of inappropriate glucagon release.

- Slowed gastric emptying, which prolongs satiety.
- Direct action on hypothalamic appetite centres, principally the arcuate nucleus, reducing hunger and energy intake.
- Insulin release is glucose-dependent, so hypoglycaemia risk is low as monotherapy.

Clinical evidence

- Strong randomised evidence — for the BRANDED product.
- STEP 1: mean weight loss near 15% of body weight at 68 weeks, against roughly 2.4% on placebo.
- SELECT: significant reduction in major adverse cardiovascular events in adults with overweight or obesity and established cardiovascular disease, without diabetes.
- Weight regain is common after discontinuation. Raise this at the start rather than letting it become a later surprise.

Absolute contraindications

- Personal or family history of medullary thyroid carcinoma (MTC)
- Multiple endocrine neoplasia syndrome type 2 (MEN2)
- Known hypersensitivity to semaglutide or any component of the formulation
- Pregnancy, breastfeeding, or planning pregnancy

Cautions

- History of pancreatitis — evaluate carefully before initiating
- Gastroparesis or other significant gastrointestinal motility disorder
- Active gallbladder disease or prior gallbladder-related surgical complications
- Severe renal impairment (eGFR below 30 mL/min/1.73 m²)
- Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
- History of suicidal ideation or active depression — no causal link has been established in regulatory review, but monitor and ask

Medication interactions

- Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
- Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
- Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
- ANAESTHESIA AND PROCEDURES: delayed gastric emptying raises aspiration risk under sedation. HOLD for 7 days (one week) before an elective procedure requiring sedation, or for whatever longer period the anaesthesia team or the surgeon's office requires - their instruction takes precedence over this one. Tell the patient to disclose GLP-1 use to any surgeon, proceduralist or anaesthetist.
- Other GLP-1 receptor agonists: do not combine. Concomitant use is not recommended.

Side effects

COMMON

- Pain, redness or swelling at the injection site
- Nausea and vomiting
- Diarrhea
- Stomach pain
- Constipation
- Low appetite
- Headaches
- Dizziness

LESS COMMON

- Pancreatitis
- Gastroparesis
- Bowel obstruction
- Gallstone attacks

Monitoring

- Weight and BMI at each visit
- Ask about blood pressure — monitored by the PCP, not measured by KORB
- Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB
- Gastrointestinal tolerance, especially through dose escalation
- Abdominal pain that is severe or radiates to the back — assess for pancreatitis
- Right-upper-quadrant pain — assess for gallbladder disease
- Mood and mental health
- Lean mass preservation — protein intake and resistance training
- Injection site and adherence

Patient counseling

Semaglutide works on the same pathway as a hormone your body already makes. It slows how quickly your stomach empties and reduces appetite signalling, so you feel full sooner and stay full longer. Nausea is the most common side effect and is usually worst in the first days after a dose increase. This is a compounded preparation rather than the brand-name product. Weight tends to come back if the medication stops, so we should talk about this as a long-term plan rather than a short course. Tell any surgeon or anaesthetist that you are taking this.

Escalation and discontinuation

HARD STOP

- Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate.
- Hypersensitivity or anaphylaxis-type reaction.
- Pregnancy identified or confirmed.

- Medullary thyroid carcinoma or MEN2 identified at any point.
- Persistent severe vomiting with dehydration or acute kidney injury.

FLAG AND REASSESS

- Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward.
- Right-upper-quadrant pain, or gallstone symptoms.
- Rapid or excessive weight loss, or loss of lean mass.
- New or worsening depression, or any mention of suicidal thoughts.
- Worsening retinopathy in a diabetic patient during rapid glycaemic improvement.
- Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely.
- Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

Chart attestation

Patient counseled on semaglutide as a GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, the need to disclose GLP-1 use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

Note. DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

PRIMARY

- E66.9 — Obesity, unspecified
- E66.01 — Morbid (severe) obesity due to excess calories
- E66.3 — Overweight

SECONDARY

- Z68.— — Body mass index (add the matching BMI code)
- Z71.3 — Dietary counseling and surveillance
- Z72.3 — Lack of physical exercise

Clinically reviewed

Reviewed and signed off by Donald Stevenson, PA-C (Director of Clinical Operations and Lead Provider) on 2026-09-06, against korb-glp1-data.js v2.13.

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Questions and escalation

- Pharmacy or shipping — Operations
- Charge codes and billing — Operations
- Clinical protocol — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute an unavailable pharmacy
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without Operations

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